

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

Congress Attendance Report — American Epilepsy Society Annual Meeting (AES) 2024

Meeting period: December 2024 | Author: Ravi Chandrasek

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

Conversation — Viraj Ashgrove, MD (Ironwood Neurology Associates)

Caught between sessions in the poster hall; the conversation was short and entirely unprompted. Referenced a session summary from a colleague in passing. Compared notes on which sessions were worth the time. Asked whether I had seen a review that circulated on the service; I had not, and offered nothing in return.

Conversation — Imani Eastmond, MD (Granite Bay Epilepsy Center)

Approached me rather than the other way around, which is worth noting. Talked through the refractory group in general terms, without any specific case. Mentioned a review that circulated on the service but did not raise any specific finding. The appeals process has improved somewhat, which came up as a general frustration.

Conversation — Saoirse Wycliffe, MD (Granite Bay Epilepsy Center)

Stopped to talk on the way out of a session block, with no agenda on either side. The practice lost the epilepsy fellow and has not backfilled, so clinic throughput keeps slipping. Formulary questions is handled by one person here, which came up as a general frustration.

Conversation — Leonie Nakagawa, MD (Ironwood Neurology Associates)

Introduced by a colleague from the same region and talked for a few minutes. Referenced a session summary from a colleague in passing. Agreed to reconnect at a follow-up appointment; nothing that needed follow-up.

Conversation — Arvind Corliss, MD (Bluestem Comprehensive Epilepsy Clinic)

We spoke standing near the exhibit aisles, briefly. Described how the refractory group is triaged now compared with before. Asked whether I would be at the meeting later this year.

Conversation — Katherine Ost, MD (Sonoran Neuroscience Institute)

A hallway conversation during the afternoon break. The composition of the refractory group has shifted over the last year or two. Asked whether I had seen a review that circulated on the service; I had not, and offered nothing in return. Mostly small talk about the program and the venue.

Conversation — Ulrike Silverthorne, MD (Silver Creek Medical Group)

We spoke standing near the exhibit aisles, briefly. Monitoring intervals during titration are set by clinic routine here rather than by protocol. Asked whether I had seen a review that circulated on the service; I had not, and offered nothing in return.

Conversation — Anouk Oyelaran, MD (Marbury Neurological Care Center)

A hallway conversation during the afternoon break. Talked through the older half of the panel in general terms, without any specific case. Compared notes on which sessions were worth the time. Asked whether I had seen a case series someone forwarded; I had not, and offered nothing in return.

Conversation — Rohan Ravenscroft, MD (Fox Hollow Neurology Partners)

Introduced by a colleague from the same region and talked for a few minutes. Noted that formulary questions varies by payer and that this is not specific to any one product. Spent some time on the pediatric-to-adult transfers and how workup decisions get sequenced.

Conversation — Hadley Rothsay, MD (Foxglove Health Neurology)

We spoke standing near the exhibit aisles, briefly. Agreed to reconnect at the following quarter; no materials requested. Benefit verification is the step that most often delays a start here. Compared notes on which sessions were worth the time.

Conversation — Orla Steinmetz, MD (Sable Creek Medical Group)

The conversation happened while waiting for a session room to clear. Referral patterns into this clinic have changed, and newly referred patients is a bigger share than it was. Left it at that, with the next visit to be scheduled. Benefit verification is the step that most often delays a start here.

Follow-ups have been logged in the interaction system.